



ACCESS, ASSESSMENT AND CONTINUITY OF CARE

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Contents

1. POLICY ON SCOPE OF SERVICES.....	3
2. POLICY ON REGISTRATION, ADMISSION & TRANSFER PROCESS.....	7
3. POLICY ON INITIAL ASSESSMENT AND REASSESSMENT OF PATIENTS	12
4. POLICY ON LABORATORY SERVICES.....	17
5. POLICY ON IMAGING SERVICES	22
6. POLICY ON SAFE PROGRAMME FOR LABORATORY & IMAGING SERVICES	24
7. POLICY ON DISCHARGE PROCESS	29

1. POLICY ON SCOPE OF SERVICES

Purpose

To define the services provided by Janapriya Hospital

Scope

The scope of this document is restricted to the services provided at Janapriya Hospital

Responsibilities and authority

The senior management of the hospital is responsible for implementing this policy.

Policy description

The services being provided are clearly defined

1. **Out Patient Services:** The hospital offers outpatient services in the following specialties:
 - General Medicine
 - General Surgery
 - Obstetrics and Gynaecology
 - Paediatrics
 - Paediatric surgery
 - Orthopedics
 - Orthopedic surgery
 - Cardiology
 - Vascular surgery
 - Neurology
 - Neurosurgery
 - Urology
 - Nephrology
 - Dialysis
 - Dental

- ENT
- Dermatology
- Endocrinology
- Day care services
- Ophthalmology
- Anaesthesiology

2. Accident and Emergency services – Medical and Surgical

3. In Patient Services:

- IP wards
- Intensive Care Units- MICU, SICU, CCU
- Operation theatres

4. Diagnostic facility:

- **Clinical Laboratory**
- **Radiology:** X-ray, Ultrasound, CT scan, MRI scan
- **Other investigations:** ECG, Echo, TMT, EEG, Spirometry, Urodynamic studies

5. Support services

- Facility Management and Safety services (Engineering services)
- Bio medical Engineering services
- Medical records
- Pharmacy services
- Physiotherapy services
- CSSD
- Ambulance services
- Housekeeping services
- Linen and Laundry services
- Security

6. Associations

- a. For advanced investigations: Divya labs
- b. Hospitals – SSM Hospital, Focus Diagnostics

Exclusions

- NICU
- Notifiable infectious diseases
- Radiation therapy
- Oncology
- Burns management

Department scope of services

- Apart from the organization scope of services, each department in the hospital shall define the department level scope of services provided
- It shall be done for both clinical & non clinical departments
- It shall be displayed in the form of a board in front of each department
- This shall be defined based on the competent staff, technology support & facilities available to take care of respective cases including both diagnostic & therapeutic services

The defined services are prominently displayed

- The services provided by the Hospital shall be displayed prominently in English & Kannada which shall be easily understood by the patients
- The details of services provided shall be displayed near the front office/entrance in the form of a permanent board
- The display of services includes Diagnostic, Ambulatory, Therapeutic and Other services
- Also the services not provided by the hospital shall also be displayed in the board as exclusions

- Tariff for the services provided shall be made available at the front office
- The services which are outsourced shall also be displayed indicating the name and the services outsourced

Staff orientation

- All the staff undergo training on the scope of services at the time of induction and also during regular trainings
- The staff trainings are maintained in the HR files by the HR department

2. POLICY ON REGISTRATION, ADMISSION & TRANSFER PROCESS

Purpose

To provide guidelines & process for registration, admission & transfer of patients

Scope

All patients undergoing treatment at Janapriya Hospital

Responsibility and authority

Front office staff, Consultants, Nursing & technical staff of Janapriya Hospital

Policy

- The hospital shall have separate protocols for registration, admission & transfer process for patients as per the scope of services provided by the hospital.
- All patients seeking services are registered and no patient shall be denied treatment due to race, color, religious or national origin.
- All staffs involved shall be oriented by periodical training.
- The hospital shall define a procedure in case the required service is not available at the hospital and such cases shall be transferred / referred for further treatment.

Policy description

A. The process addresses registering and admitting outpatients, inpatients & emergency patients

The hospital shall have separate protocol for registration and admission of out patients, in-patients and emergency patients including new borns

1. Registration process

- The front office staff shall collect patient personal information, services required, consultant details & nature of services- outpatient/inpatient services

- The front office staff then generates Registration number. This is generated at the time when the patient first visits the hospital and this remains unique for that patient for subsequent visits. This ensures safety, security and continuity in care services.
- During subsequent visits by the same patient, the Registration number generated during the first visit remains same and all the billing for OPD services happen with this.
- For IP admissions, the Registration number remains the same but the Inpatient (IP) number is given separately for each admission.
- For IP admission each time a new IP number is generated
- A separate registration number is generated to all the newborns who are given birth at Janapriya Hospital apart from the registration number generated to the mothers
- The process of generation of registration number remains the same as for adults
- The insurance details shall also be collected in case of inpatients
- The billing process also shall be carried out in the front office

2. Admission process

- The hospital shall have a policy that no patient shall be denied admission due to race, colour, religion etc. The sources of admission are from
 - a. OPD
 - b. Emergency room
 - c. Direct / pre-scheduled admission

The admission protocol shall include:

- Patient admission- The new IP number shall be generated each time the patient gets admitted to the hospital (specific for each admission), apart from the registration number.
- Depending on the clinical condition, urgency and required bed category, the admission is done with patient / family member's consent.
- Admissions are accepted 24 hours a day, 7 days a week, irrespective of any holidays.
- All staff involved shall be oriented to the policies and procedures by periodical training atleast once in a year.

Emergency patients can be:

- a. The patient requiring immediate medical or surgical intervention shall be admitted through the Emergency Department.
 - b. All Medico legal cases and trauma/road traffic accident cases will be guided to the emergency department and admitted
- The hospital shall define the scope of services it can provide and displays the same in the hospital
 - The staff especially the front office shall be made aware of the scope of services along with the exclusions
 - The staff shall also be oriented to contact the Administrator in case of any clarifications
 - However, in cases of emergency, the hospital shall first stabilize the patient and then depending the clinical condition shall either admit or transfer the patient to other hospital
 - The hospital shall develop a protocol to address prioritization of patient to identify for patients needing priority attention, based on clinical condition.
 - This shall be followed across all service areas/ clinical settings (OP/IP/ Emergency / diagnostics services)
 - The staff handling these activities shall be oriented in following this protocol.
 - The staffs / care providers shall be made well aware of this protocol during initial and regular trainings. They are provided necessary information in identifying such patients including applicable laws and priority management protocol based on clinical condition.

B. The process addresses mechanism for transfer or referral of patients who do not match the organizational resources.

- The hospital shall have defined procedure for
 1. Transfer in
 2. Transfer out / referral of stable / unstable patients
- The patients are transferred in form other hospitals based on the scope of services and facilities available at the hospital. The relevant medical records of the patients with all the details are collected during the transfer in process
- The patients are usually transferred out of Janapriya Hospital to appropriate healthcare facility either from A & E department, IP ward / ICU & for diagnostics services
- The treating consultant shall decide the transfer out based on the clinical condition of the patient and beyond the scope of services provided by the hospital
- The following protocol shall be followed:
 - a. Before transfer
 - i. Communication with family members and their written consent shall be taken
 - ii. Identify the receiving hospital and ensure availability of bed
 - iii. The treating consultant of transferring hospital will speak to the consultant of receiving hospital to brief on the patient condition
 - iv. Transportation arrangement and trained transport team
 - v. All required medical records with transfer summary is handed over to the transport team
 - vi. Doctor accompanies in the ambulance during the transfer of unstable patient
 - b. During Transfer
 - i. Ensure availability of Doctor and team of Trained personnel (Nurse/Doctor)
 - ii. Adequate resources (Life saving equipments and emergency medication) are available and administered only on need basis

- iii. Vitals are monitored continuously and regular communication at receiving hospital is to be ensured, including any changes in the condition of the patient during transit.
- iv. Physician's orders followed all through the time of transfer.

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3. POLICY ON INITIAL ASSESSMENT AND REASSESSMENT OF PATIENTS

Purpose

To ensure that all patients in the hospital undergo an established documented initial assessment & reassessment in a structured manner

Scope

All patients who are provided care as in-patients / daycare, out-patients & emergency patients at Janapriya Hospital

Responsibility and authority

The overall responsibility for implementation would be with the Chairman and Consultants

Procedure

The organization defines the content of assessments for outpatients/inpatients/emergency patients

- The hospital shall develop standardised format for conducting initial assessment across OPD, IPD, day care and Emergency patients
- The assessment format shall have vital parameters, patient present complaints, vital signs, and systemic examination
- The assessment forms can be department specific i.e for OPD, day care, ICU, OTs, Labour room or for a specific group of patients like Pregnant, Pediatric etc
- There shall be separate assessment forms for Doctors and nurses
- The OPD assessment form for Doctors shall have a brief history, vitals monitoring, examination and treatment/investigations advised based on history & examination. The treatment shall include the medications orders and the next follow up date for reassessment of the patient
- The IPD initial assessment form shall have:

- Patient presenting complaints
- Past illness – medical/surgical
- Any known allergies
- Family history / personal history
- Nutritional screening/assessment
- Patient examination
 - Vital parameters
 - General physical/ systemic examination
- Provisional diagnosis /list of diagnostic investigations
- Care plan
- The initial assessment is done to derive the care plan of each patient and is recorded in the medical records

Documented Care plan

- The results of the initial assessment findings provide inputs for treating team and this shall get reflected as “plan of care” / “care plan”, which shall include preventive aspects of care as well.
- Results of initial assessment are monitored in subsequent re-assessments. This is also documented and included in the patient’s plan of care
- Care plan of the patient shall include special need identification for special category of the patient which includes – extremes of age, restricted mobility, nursing need continuity and rehabilitation.
- Patient and family education and instruction shall be provided which are essential components of the discharge plan, which forms the later part of re-assessment.

Authorized personnel for conducting assessments & reassessments

- The hospital shall identify authorised personnel across various departments for performing an initial assessment & reassessment as per the credentials & privileges.
- The following are authorized to perform assessments:
 - Doctors including consultants, senior and junior Medical officers of various departments

- Registered Nurses
- The hospital shall develop a policy to define the process of initial assessment and completion of the documentation for all the admitted patients.
- The time frame shall be defined from the time the patients are registered or arrived at the specific department to till the time the initial assessment is completed
- The hospital shall have a protocol to complete the assessment as soon as possible. The maximum time for completing the documentation process is 24 hours but can be completed earlier depending on the patient's condition
- The hospital shall have a policy that the initial assessment for all In-patients needs to be completed and documented within 24 hours, depending on the patient condition.
- It shall be done as soon as possible and the maximum time for completing the documentation process is 24 hours.
- However, if the admitted patient is from OPD / A & E department then a detailed assessment shall be carried out and documented by that department.
- The IP record shall indicate the linking comment against the time in the ward upon admission.

Reassessment of patients

- Reassessments are documented in the following records:
 - b. Medical Staff:
 - i. Progress notes
 - ii. Pre/Post anaesthesia notes
 - iii. Consultation reports
 - iv. Operative notes
 - v. Discharge Summary
 - vi. Outpatient notes (Prescription)
 - c. Nursing Staff
 - i. Nurses progress notes

- ii. TPR sheets, vitals monitoring records
 - iii. Intake / output records
 - iv. Medication records
-
- The frequency of re-assessment differs based on the departments – ICU / Ward / Day-care and based on the progress & clinical condition of the patient
 - At least one re-assessment per day shall be conducted by the treating doctor
 - The reassessment shall be documented in progress notes/ treatment chart/doctors order by the doctors
 - The nursing staff shall record the reassessment findings in treatment chart, vital signs monitoring records, nurse's progress notes, procedure notes etc.
 - The outcome of the re-assessment is to revisit the 'care plan' defined at the time of initial assessment.
 - The treating doctor and nurses shall document the reassessment findings in the medical record which shall include vital parameters, systemic examination, and medication order
 - This is recorded in the chronological order to monitor the patient's progress.
 - The reassessment documentation shall be specific in nature and not generalized and shall clearly indicate the patient's present condition
 - The re-assessment shall focus on the response to the treatment, 'care plan' and further towards the discharge process.
 - This will include measuring the vital parameters, airway, circulation, neurological status, and other signs to address the concern of the staff/ patient/family member to each patient
 - The staff are trained to communicate the information of patient conditions with the warning signs and signs to watch for to the treating doctor/ team member at the earliest for prompt treatment by the Doctor
 - This shall be documented in the medical files
 - The treating doctor plans the discharge of the patient based on the reassessment findings

- It shall be a multi disciplinary approach and the opinion of all care providers are taken into consideration by the treating doctor to plan the discharge of the patient
- A detailed re-assessment is carried out during the time of discharge including vitals and systemic / focus examinations which shall be documented in the medical record
- The plan of discharge starts from the time of admission and is documented in the care plan.
- The outpatients after consultation shall be advised on re-assessment as a follow-up note, including a specific date / interval and this shall be documented in OP record which includes:
 - Medication advice
 - Revisit period or in case of any untoward symptoms or occurrence of warning signs demanding early review (especially post – procedure)
 - Review with diagnostics test reports and further, to change the care plan, if need be
- The follow up advice is mandatory for all patients visiting for OP consultation except for the clinical condition where further review is not required and for patients seeking second opinion

4. POLICY ON LABORATORY SERVICES

Purpose

To help in diagnosis, prognosis, treatment and prevention of diseases

Scope

All patients treated at Janapriya Hospital

Responsibility

Consultant, laboratory technician

Procedure

A.Scope of laboratory services

- The scope of laboratory services shall be in line with the defined services offered by the hospital
- The resources and equipment planning shall also be in line with hospital scope of services
- The lab services and the timings shall be displayed with proper signages in the premises
- The laboratory tests with the rate list shall be displayed in the department
- Also the tests outsourced to other hospitals and also the turnaround time for the tests shall be clearly displayed
- The Hospital ensures adequate space and easy access to the laboratory, taking care to demarcate areas of danger and indication of 'No-entry' for patients. Also signage for phlebotomy area, toilets, display of menu of tests, and other sample collection facilities is clearly indicated
- The laboratory personnel are trained to address concerns of patient comfort like wheel chair and clear communication system.

- The department is appropriately equipped as per its services, the reagents, kits, and inventory management for routine, emergency and specialized laboratory tests are made available. There shall be a checklist to manage the inventory control.
- The hospital shall ensure that the laboratory department shall have doctors from different branches of laboratory medicine and trained technicians (senior and junior staff).
- The specific job descriptions shall be defined for each category of staff
- The staffs are adequately trained with training records maintained in their respective personal file.
- The staffs are trained when new technology (equipments) is introduced or if there is change in the job role.

B. Procedures guide collection, identification, handling, safe transportation, processing and disposal of specimens.

- The hospital shall ensure that the laboratory maintains an SOP to guide the pre-examination, examination and post-examination workflow adhering to the statutory requirements and good clinical laboratory practices guidelines.
- There shall be demarcated areas for specimen collection (phlebotomy area), pre analysis, analysis and post analysis (reporting) area
- The department shall follow the following:
 - The investigations are ordered based on the medical condition and shall only be advised by the treating doctor.
 - General checklist shall be followed during sample collection and adequate volume of sample shall be collected
 - The OPD samples shall be collected in the phlebotomy area by the lab technician and in patient samples are collected by the nurse/technician in the wards/ICU & later sent to lab for analysis
 - Disposal of sharps/disposables as per protocol

- Patient shall be identified before sample collection using registration number & lab number
- The samples are handled in a safe manner by avoiding contamination and breakages.
- Traceability of samples shall be followed
- The appropriate method of disposal for different types shall be followed

C. Laboratory results are available within a defined time frame and critical results are intimated immediately to the concerned personnel.

- The department shall have a policy document the Turn Around Time (TAT) and use as a benchmark to monitor laboratory effectiveness.
- TAT shall be defined at various intervals:
 - From order placement to specimen collection
 - Transportation time from the site of sample collection to receipt at the laboratory
 - Pre-analysis time (Sample movement within the laboratory to its specific test section, and steps in preparation)
 - Analysis time (Processing time)
 - Post analysis (Time after completion of analysis until results are verified and conveyed to the clinical team as a result)
- TAT shall be presented in the Management review meeting for approval and will be reviewed at regular intervals (yearly once).
- The list of ‘critical values for tests’ shall be displayed in the department.
 - The laboratory shall have a process to notify critical results verbally to the concerned doctor and this communication shall be documented.
 - The policy on critical alert intimation is also applicable to outsourced labs as well
 - The outsourced staff shall take the responsibility to inform the hospital lab department in charge at times of critical values when found
- The staff shall be trained at regular intervals and re-training is done as needed

D. Laboratory personnel are trained in safe practices and are provided with appropriate safety equipment/ devices.

- Staffs working in the laboratory department shall be trained in general and specific laboratory safety practices.
- Laboratory safety training protocol includes:
 - General principles of safety related to infrastructure, equipment and personal protection measures.
 - Safety policies and protocols related to fire, hazardous materials, biomedical waste management and electrical safety.
 - Training on safety devices
 - Personal protective equipment (PPE)
 - The immunization records are maintained in the personnel file for regular follow up.

Equipment calibration

- The hospital shall have a protocol for calibration and maintenance of all equipments (major and minor equipments)
- The records for all maintenance process like AMC, PMC, downtime, replacement of spare parts shall be documented in the equipment register in each department
- The quality department shall maintain a calibration calendar and maintenance details
- The certificates of calibration shall be maintained separately and stickers shall be placed on equipments
- Daily ,weekly and other maintenances (AMC, PMC, Break down records, replacement of spare parts etc) shall be maintained in the department

- All the equipments in the laboratory shall be labeled with unique ID and details like vendor, date / year of purchase, date of calibration, AMC / PMC and due date.
- All equipments (minor & major equipments) which has pressure and temperature gradient shall be calibrated annually
- Regular equipment maintenance as per manufacturer's recommendation shall be carried out and documented including breakdown and spare parts replacement notes

E. Laboratory tests not available in the organization are outsourced to other organizations based on their quality assurance system

- The hospital shall define a list of tests not being done in the laboratory.
- The hospital shall have agreement (MoU) with referral laboratory which shall meet the quality assurance requirements.
- The hospital shall in random visit the outsourced labs and check for the quality systems followed and the visit reports shall be documented
- Also few samples are sent to other certified labs to check the correctness and reliability of the tests performed at the outsourced labs. The same shall be documented

5. POLICY ON IMAGING SERVICES

Purpose

To define the scope of radiology department services

Scope

All radiology tests done at Janapriya Hospital

Responsibility and authority

All staff

Procedure

A. Scope of the imaging services shall be commensurate to the services provided by the organization

- The scope of the radiology department shall be as per the services offered by the hospital
- The department shall provide the following services:
 - X-Ray
 - Ultra sound
 - CT scan
 - MRI Scan
- The trained resources shall be available to operate the equipments and report on them.
- The hospital shall have adequate staff with appropriate qualifications and training to perform and supervise investigations.
- The job description shall be defined and regulatory compliances are ensured.
- Interpretation of the test results shall be done by authorized personnel (as per the credentialing and privileges) of the hospital.
- The staffs shall be trained in report generation process including critical value reporting when required

B. Infrastructure, Human resources & signages

- The department is designed and built to meet the regulatory requirements and accordingly the physical infrastructure and equipments shall be planned
- The compliances and necessary approvals are obtained from AERB right from building stage to commissioning stage.
- All the important signages as per the regulatory requirements are displayed in the department and also in outside wherever patient movement takes place
- The Pre-Natal Diagnostic Techniques (PNDT) regulations, along with other signages shall be displayed in the department
- The hospital shall put proper sign boards /postings / red bulbs when department is working to indicate danger and warning to people not to unnecessarily move in areas demarked.
- Availability of a hygienic environment with basic systems like uninterrupted power supply, adequate lighting, water supply, ventilation and controlled temperature and humidity
- The protocols address the control of waste disposal, noise and air pollution in adherence to statutory regulations.

C. Imaging results are available within a defined time frame and critical results are intimated immediately to the personnel concerned

- The department shall develop a list of critical results for each test. This shall also includes those results of outsourced investigations as well
- The department shall define the TAT (turn around time) for all imaging services
- All staff shall be made aware of the same through regular training.
- The department shall develop a policy to indicate the critical values / results and the staff are trained to intimate the same to the treating Physician.
 - This shall be documented in the critical value reporting register.

- The radiology department shall a list of tests not being done and these shall be outsourced to outside centres
- The hospital shall have a defined MOU, with defined the quality assurance systems.

D. Imaging personnel are trained in safe practices and are provided with appropriate safety equipment/ devices.

- The hospital shall ensure the staff are trained on all safety precautions
- Imaging personnel and patients exposed to radiation shall be protected from its harmful effects.
- Staffs are provided with TLD badges and Lead aprons to be used by doctors/ technicians during radiation exposure examinations.
- The hospital shall have a quality assurance program as per AERB regulations
- The procurement equipment and other equipment shall have legal binding documents between the organization and the licensed authorized vendor, for the supply of NOC validated/ type approved equipment.
- The department shall have a protocol to check the calibration and maintenance for all equipments as per the AERB guidelines and in compliance with manufacturer's recommendations
- A calibration calendar shall be maintained for calibration and maintenance of equipment.
- Radiation protection devices including PPE shall be provided to department personnel

7. POLICY ON SAFE PROGRAMME FOR LABORATORY & IMAGING SERVICES

- The Laboratory safety program / plan will include:

- General laboratory safety practices – like dress code, discipline, good technique and good housekeeping practices, use of PPE, Employee immunization, Staff awareness and regular training.
- Installation / Use of Safety devices include:
 - a. Eye wash facility
 - b. Fire extinguisher
- Infection control measures – like biomedical waste management, decontamination procedures, Universal and standard precautions, exposure to infectious agents and their bio-safety level
- Chemical hazards and management – like handling, storage, disposal of hazardous chemicals, use of MSDS sheets, spills and exposure management
- Fire safety - Identify sources of fire, preventive measures with fire safety equipments, fire safety protocol
- Electrical safety – causes, precautions and emergency handling protocols
- Mechanical safety – Safety checking and safe handling of equipments and instruments handling, handling of glassware containers, sharps, bottles etc
- Accidental exposure – Corrective and preventive measures to avoid accidental exposures to blood or body fluids, puncture etc
- Disaster preparedness – Department preparedness and organization preparedness to disasters either natural / man made.
- The laboratory safety protocols
 - Shall be in line with the hospital general safety principles:
 - Applicable guidelines like OSHA, Biomedical waste services, MSDS, Hazard identification and management, NACO, RNTPC is followed as common protocol
 - General Infection control measures, Disease transmission prevention protocol
 - General safety policies like – Electrical, Mechanical
 - Staff awareness, training, Immunization, use of PPE
 - Emergency management – Emergency codes and activation
 - Fire safety protocol

- Equipment safety measures
- Risk assessment and management protocol
- Accidental injury and management
- Disaster management protocol
- Safety audit - includes infrastructure, equipments, staff awareness and training, systems and processes across departments.
- Laboratory waste disposal practices are based on
 - Defined regulatory guidelines
 - Recommended measures to handle infectious agents
 - Decontamination process
 - Hazardous material disposal practices including labelling and treatment
- Hospital Safety committee
 - The laboratory safety program is part of Janapriya Hospital safety program and the same committee members shall address the concerns and agenda for clinical laboratory department
 - Perform risk assessment and analysis towards providing necessary CAPA
- Hospital training program includes:
 - General principles of safety related to infrastructure, equipment and personal protection measures.
 - Safety Policies and Protocols related to fire, hazardous materials, bio medical waste management, electrical safety,
 - Awareness of applicable guidelines and standard precautions against infectious agents.
 - Department specific safety concerns and reporting systems.
 - Good laboratory practices
 - Disposal of waste and Infection control practices
 - Training on MSDS
 - Sharp management
 - Spills management
 - Equipment handling, storage, transportation and use.

- The hospital shall provide adequate safety devices and are available within the department like PPE, dressing materials, disinfectants, fire extinguishers etc its employees
- Policies for employee immunization are defined and followed regularly. The immunization records are maintained in the personal file for regular follow up.

Imaging safety programme

- The department defines a protocol to identify such patients at risk and develop a protocol to address them. These groups will need extra attention, due to their vulnerability. This includes:
 - Patients who are in the child bearing age / pregnant
 - Pediatric group
 - Geriatric group and
- Safety practices across department functions in compliance with regulatory requirements. This shall address the safety exposure rate for patients and to the environment against each type of the procedure and as per the clinical requirement and clinical condition of the patient
- A certified Radiation safety officer shall be appointed and designated accordingly.
- All imaging personnel as well as those who occasionally use radiation equipments are trained adequately in radiation safety measures.
 - The safety precautions followed for patients are indicated to their family members and are guided to stay away from the defined restricted area
 - A prior informed written consent is obtained for procedures which involve higher risk of imaging. The side effects (like allergy) and the procedure is explained to the patient and their family members.
 - The handling and disposal procedure should be in line with the defined guidelines of AERB and should be traceable.

- Personnel monitoring badges shall be available for all staff exposed to ionizing radiation, from Authorized regulatory authority. TLD (Thermoluminescent Dosimeter), shall be used. This is stored in a radiation free area. The TLD readings are evaluated and reported at regular intervals.
- Lead aprons shall be used by doctors/ technicians during radiation exposure examinations.
- Lead eyeglasses with wrap around eye shields
- Lead aprons shall be examined for creases, cracks and rupture

Imaging signages

- There shall be a provision of red-light bulbs outside each radiation facility. This is to indicate that a radiation emitting procedure is on and to prevent trespassing and disturbance during that phase
- Usage and display of radiation symbols, radiation hazards and radiation x-ray hazards and radiation sealed sources as per Regulatory specifications.
- Imaging signage and cautionary signages, warn people about the radiation zones/ areas. These are prominently displayed in all appropriate locations in the form of bilingual display boards.

8. POLICY ON DISCHARGE PROCESS

Purpose

To ensure that there is a smooth and documented discharge process

Scope

All patients undergoing treatment at Janapriya Hospital

Responsibility and authority

All staff members involved in patient care of Janapriya Hospital

Procedure

Process addresses discharge of all patients including Medico-legal cases and patients leaving against medical advice.

- The hospital shall have a discharge process for all patients getting discharged including Medico-legal cases and patients leaving against medical advice.
- The treating doctor is the final authority in declaring the patient medically fit for discharge which shall be done during the periodic and regular reassessments.
- In medico legal cases, the police shall be informed at the time of discharge so that they can complete or guide the patient and or the family about the formalities.
- The same protocol shall be followed for all types of discharges

A. A discharge summary is given to all the patients leaving the organization (including patients leaving against medical advice).

- Discharge consultation shall be conducted by the doctor before planning for discharge to assess the condition of the patients
- The hospital shall provide a discharge summary to all patients getting discharged including LAMA

- The treating doctor or one of his team members shall sign the discharge summary
- The patient or the family member acknowledges the receipt of the summary in the document handover register.
- In addition to the summary, the **original diagnostic reports** shall also be given to the patient at the time of discharge. The **photocopied reports** are generally handed over to the patients in cases of insurance discharges.
- 2 copies of discharge summary shall be prepared, one is provided to the patient and the other one is included in the patient's records in the hospital.
- Patients discharge status (Date and time) is entered in the Admission and Discharge register.

B. Discharge summary contains the reasons for admission, significant findings, investigation results, diagnosis, procedure performed (if any), treatment given and the patient's condition at the time of discharge.

- The discharge summary shall contain patient's personal and demographic details along with the treating doctors details
- Additionally it shall contain the following points:
 - Reasons for admission to the hospital
 - Clinical findings on examination
 - Final Diagnosis
 - Patient's condition at the time of discharge
 - Results of the investigation reports
 - Procedures performed
 - Medications administered

C. Discharge summary contains follow-up advice, medication and other instructions in an understandable manner.

- The discharge summary shall incorporate the following details:
 - Follow up advice- The treating doctor shall decide on the day and time to come for the follow-up
 - Medicines to be taken- This shall include the medications that the patient must continue to take at home as well.
 - Other instructions – diet, physiotherapy, care of the wound, exercises, monitoring, precautions to be taken, preventive aspects etc preventive aspects
 - DO's and Don't's are also clearly mentioned in the summary
- This advice shall be explained in the language understood by the patient and the family. Abbreviations shall be avoided as far as possible.

D. Discharge summary incorporates instructions about when and how to obtain urgent care.

- The hospital shall list out the conditions when, how and where the patient has to obtain urgent care in case required
- The hospital shall list down the probable conditions regarding when to obtain urgent care after discharge.
- The hospital contact numbers shall be mentioned in the discharge summary.
- These shall be explained to the patient and the family in the language understood by them either by the treating doctor or one of his team members or the nurse present in the shift.

E. In case of death, the summary of the case also includes the cause of death.

- The discharge summary shall mention the cause of death after it is ascertained.
- It shall be written in the final diagnosis and in the course in the hospital. The primary and intermediate causes shall also be mentioned in the summary.

- A copy of death intimation form shall be prepared & signed by the concerned doctor/CMO/Anaesthetist and the same shall be maintained in the medical file
- If the cause of death is not ascertained, a clinical autopsy or a post-mortem shall be requested either by the treating doctor or the relative. An autopsy is mandatory in medico-legal cases.

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